



Palliative Care



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Introduction

Chronic and non-infectious diseases have been increasing around the world, resulting in an increasing demand for palliative care for patients with life-threatening conditions and the elderly population throughout the world. Each year, an estimated 40 million people are in need of palliative care, 78% of them live in low- and middle-income countries. Due to this increasing need for palliative care, the World Health Organization and the Worldwide Palliative Care Alliance proposed that palliative care can be provided in every locally available condition: at home, in hospitals, outpatient clinics and care centres, or wherever the patient is being cared

Palliative care is a specialized form of care that aims to enhance the quality of life of patients and their families who are faced with serious illness. It focuses on increasing comfort through prevention and treatment of distressing symptoms. In addition to expert symptom management, palliative care focuses on clear communication, advance planning, and coordination of care. The terms "palliative care" and "hospice care" cannot be used interchangeably. Hospice is only one type of palliative care that can be provided in the last six months of life. Traditional palliative care can be offered any time in the course of an illness and ideally should begin at the time of diagnosis.

WHO Definition of Palliative Care

Palliative care is an approach that improves the quality of life of patients and their families facing the problem associated with life-threatening illness, through the prevention and relief of suffering by means of early identification, assessment, treatment of pain and other problems(physical, emotional, psychosocial and spiritual).

Objectives of Palliative care:

1. Provides relief from pain and other distressing symptoms;
2. Affirms life and regards dying as a normal process;
3. Integrates the psychological and spiritual aspects of patient care;
4. Offers a support system to help patients live as actively as possible until death;
5. Offers a support system to help the family cope during the patients illness and in their own bereavement;
6. Uses a team approach to address the needs of patients and their families, including bereavement counseling, if indicated;
7. Enhance quality of life and may also positively influence the course of illness.

Types of diseases requiring Palliative Care

Palliative care treats people suffering from serious and chronic illnesses including

- ❖ Cancer(malignancies)
- ❖ Cardiac disease such as Congestive Heart Failure (CHF),
- ❖ Chronic Obstructive Pulmonary Disease (COPD),
- ❖ Chronic renal failure,
- ❖ Chronic liver disease
- ❖ Leukemia
- ❖ neurodegenerative disorders and dementia
- ❖ Alzheimer's disease
- ❖ Parkinson's disease
- ❖ HIV/AIDS
- ❖ Lateral Sclerosis (LS).

Palliative care relieves the symptoms of these diseases, such as pain, shortness of breath, fatigue, constipation, nausea, loss of appetite depression, confusion, difficulty sleeping and treatment side effects

Domains of Palliative Care

- Structure and Processes of Care
- Physical Aspects of Care
- Psychological Aspects of Care
- Social Aspects of Care
- Spiritual Aspects of Care
- Cultural Aspects of Care
- Care of Imminently Dying

Structure and Process of Care

- Begins with a comprehensive assessment and a care plan that is consistent with a patient's values and goals
- Advance Care Planning: patient and family treatment goals are clearly documented
- The primary non-medical needs expressed most frequently include: a need to express emotional pain, a need to explore spiritual pain and a need for practical financial and legal help

Physical Aspects of Care

- Assessment should focus on relieving symptoms, improving/ maintaining quality of life and functional status
- Symptoms may include pain, shortness of breath, fatigue, nausea, constipation, etc.
- Lack of assessment is the most common cause of unrelieved pain
- Care is delivered in a manner that is patient centered as defined by the patient's wishes

Psychological and Psychiatric Aspects of Care

- Psychological status needs to be assessed and managed
- Watch for signs of family members struggling with psychological issues
- Programs and resources should be available to patients and families based on assessed need for services
- Process for appropriate referrals:
 - Directly
 - Through Consultation
 - Specialist Referral

Social Aspects of Care

- Social assessment should address environmental and social factors, including, but not limited to:
 - Social support network
 - Financial barriers
 - Access to care (e.g. transportation, medications)
- Family Meeting: powerful clinical tool for completing the comprehensive assessment and planning process
- Referrals to local/community service providers

Spiritual, Religious and Existential Aspects of Care

- Spirituality is a multifaceted, multidimensional human experience that includes religious and nonreligious factors
- Care Team members must acknowledge their own spirituality
- Offer support of spiritual counselor: priest, pastor, chaplain, rabbi, imam, or other religious leader
- Faith, Importance, and Influence, Community and Application (FICA) assessment

Cultural Aspects of Care

- Racial and ethnic minorities experience persistent health care disparities
- Cultural origins influence the way patients and health care providers

think about palliative and end of life care

- Respect values, beliefs, and traditions related to health, illness, family caregiver roles and decision making

- Incorporate culturally sensitive resources and strategies into the plan of care

- Remove barriers to communication by ensuring that linguistic needs are met

Care of Imminently Dying

- Whenever possible, early access to hospice care should be facilitated
- Place particular emphasis on days leading up to and just after death of the patient
 - Ensure patient receives adequate management of pain and other symptoms
 - Avoid inappropriate prolongation of dying
 - Address spiritual and cultural needs
- Signs and symptoms of impending death are recognized and communicated to

patients and families

- Provide support and education to the family
- Assist in making critical decisions
- Relieve possible burdens imposed on loved ones
- Develop post-death care and bereavement follow up plan

Key elements of palliative care:

- ❖ **Patient population:** The population served includes patients of all ages experiencing a debilitating chronic or life-threatening illness, condition, or injury.
- ❖ **Patient and family centered care:** The uniqueness of each patient and family is respected, and the patient and family constitute the unit of care. The family is defined by the patient or, in the case of minors or those without decision-making capacity, by their surrogates. In this context, family members may be related or unrelated to the patient; they are individuals who provide support and with whom the patient has a significant relationship. The care plan is determined by the goals and preferences of the patient and family, with support and guidance in decision-making from the healthcare team.
- ❖ **Timing of palliative care:** Palliative care ideally begins at the time of diagnosis of a life-threatening or debilitating condition and continues through cure or until death and into the family's bereavement period.

- ❖ **Comprehensive care:** Palliative care employs a multidimensional assessment to identify and relieve suffering through the prevention or alleviation of physical, psychological, social, and spiritual distress.
- ❖ **Interdisciplinary team:** Palliative care presupposes indications for, and provision of, interdisciplinary team evaluation and treatment in selected cases. The palliative-care team must be skilled in care of the patient population to be served. Palliative-care teams may be expanded to include a range of Professionals based on the services needed. They include a core group of professionals from medicine, nursing and social work, and may include some combination of volunteer coordinators, bereavement coordinators, chaplains, psychologists, pharmacists, nursing assistants and home attendants, dietitians, Speech and language pathologists, physical, occupational, art, play, music, and child-life therapists, case managers, and trained volunteers.
- ❖ **Attention to relief of suffering:** The primary goal of palliative care is to prevent and relieve the many and various burdens imposed by diseases and their treatments and consequent suffering, including pain and other symptom distress
- ❖ **Communication skills:** Effective communication skills are requisite in palliative care. These include developmentally appropriate and effective sharing of information, active listening, determination of goals and preferences, assistance with medical decision-making, and effective communication with all Individuals involved in the care of patients and their families.

- ❖ **Skill in care of the dying and the bereaved:** Palliative care specialist teams must be knowledgeable about prognostication, signs and symptoms of imminent death, and the associated care and support needs of patients and their families before and after the death, including age-specific physical and psychological syndromes, opportunities for growth, normal and aberrant grief, and bereavement Processes.
- ❖ **Continuity of care across settings:** Palliative care is integral to all healthcare delivery system settings (hospital, emergency department, nursing home, home care, assisted living facilities, outpatient, and nontraditional environments, such as schools). The palliative care team collaborates with professional and informal caregivers in each of these settings to ensure coordination, communication, and continuity of palliative care across institutional and homecare settings. Proactive management to prevent crises and unnecessary transfer are important outcomes of palliative care.
- ❖ **Equitable access:** Palliative care teams should work toward equitable access to palliative care across all ages and patient populations, all diagnostic categories, all healthcare settings including rural communities, and regardless of race, ethnicity, sexual preference, or ability to pay.

The Palliative Care Team



This team includes:

- Palliative care physicians
- Specialists or general practitioners
- Nurses
- Nursing assistants or home health aides
- Nutritionists
- pharmacists
- Social workers
- Spiritual counselors
- volunteers
- Chaplains
- Physical, occupational, and speech therapists

Where is palliative care provided?

Palliative care can be provided:

- At the home (with support from specially trained staff)
 - In a hospital
 - In nursing home

- In a hospice day center

Palliative care at home has many documented benefits. These include the following:

- ❖ Reduced burden of symptoms on the patient and their family
- ❖ Increased patient and family compliance to care and treatment
- ❖ Improved recovery
- ❖ Improved patient and family satisfaction
- ❖ Reduced hospital, intensive care and emergency room stays
- ❖ Reduced hospitalisations
- ❖ Preserving family integrity
- ❖ Enabling the patient to live in their home setting
- ❖ Preventing the social isolation of the individual
- ❖ Providing the highest possible autonomy
- ❖ Allowing the patient to participate in family and social activities and hobbies
- ❖ Reduced risk of infection
- ❖ Allowing a more personal care
- ❖ Reduced stress for the patient and their family
- ❖ A lower short-term cost
- ❖ Uninterrupted health care from diagnosis to recovery
- ❖ Improved quality of life